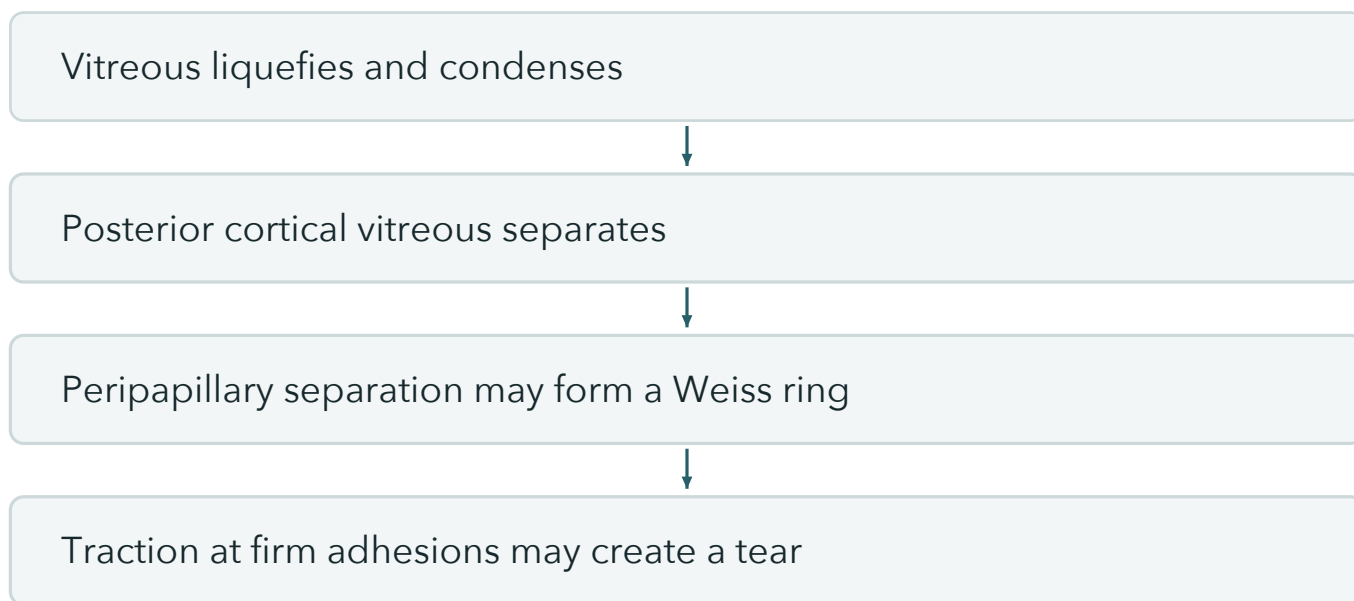


PVD explains symptoms but does not end the examination

New flashes and floaters require a search for a retinal break or haemorrhage.

Vitreous separation and a retinal risk



Clinical meaning

Floaters are vitreous shadows; flashes reflect retinal stimulation by traction. A Weiss ring with an attached retina supports PVD, but dilated examination must still seek a tear, haemorrhage or detachment. Haze or a curtain needs particular attention. Advise urgent review for new or worsening symptoms.

Blood can conceal the diagnosis behind it

When vitreous opacity prevents a retinal view, assess the hidden retina with B-scan.



Vitreous haemorrhage in the left panel obscures the retinal vessels; the right panel provides a clearer comparison.

Clinical meaning

Vitreous blood can follow trauma, a tear with acute PVD or fragile proliferative neovessels. The red reflex may be reduced despite a clear lens. If blood obscures the retina, B-scan assesses the hidden posterior segment. Treat the underlying cause; persistent non-clearing blood may lead to vitrectomy consideration.